

Recent Tier 1 papers — not yet indexed (needs review)

• **Cycle:** 2026-07-03 | **Window:** 2026-06-19 to 2026-07-03

• **Tier 1 journals scanned; items found:** 2

These are recent papers from the approved **Tier 1** journals that the strict, MeSH-verified digest does not yet catch because PubMed has not finished indexing them (no MeSH terms or publication types assigned yet). They are matched by journal + publication date + disease name (and, unless the broad option was used, phase II/III-trial or guideline wording in the title/abstract). Case reports/series and preclinical/animal studies are filtered out. They are **not** AI-appraised, and the study type is inferred from wording, not verified. Treat as a heads-up list to scan manually; verify each against the source.

Diffuse Large B-Cell Lymphoma

Tafasitamab plus lenalidomide and R-CHOP versus R-CHOP for first-line treatment of patients with high-risk diffuse large B-cell lymphoma (frontMIND): a global, phase 3, randomised, double-blind, placebo-controlled trial.

Journal: Lancet (London, England) | **Date:** 2026-Jun-20 | **PMID:** 42217458 | **DOI:** 10.1016/S0140-6736(26)00866-4

Publication types: Journal Article, Randomized Controlled Trial, Clinical Trial, Phase III, Multicenter Study

Abstract:

BACKGROUND: Approximately 40% of patients with high-risk diffuse large B-cell lymphoma (DLBCL) are not cured with first-line R-CHOP (rituximab, cyclophosphamide, doxorubicin, vincristine, and prednisone or prednisolone). We aimed to investigate the addition of tafasitamab (an Fc-enhanced anti-CD19 monoclonal antibody) and lenalidomide to R-CHOP (tafa-len-R-CHOP) in patients with high-risk aggressive B-cell lymphomas.

METHODS: frontMIND is a phase 3, randomised, double-blind, placebo-controlled study conducted at 298 centres in North America, South America, Europe, and the Asia-Pacific region. Patients aged 18-80 years with previously untreated, high-intermediate-risk or high-risk DLBCL or high-grade B-cell lymphoma (HGBL) were randomly allocated (1:1), stratified by International Prognostic Index (IPI) or age-adjusted IPI and geographical region, to receive six 21-day cycles of standard R-CHOP (rituximab 375 mg/m

FINDINGS: Between May 11, 2021, and March 2, 2023, 1229 patients were screened, among whom 899 were randomly allocated: 448 (50%) to the tafa-len-R-CHOP group and 451 (50%) to the R-CHOP group. At the time of primary analysis (median follow-up 35.2 months [95% CI 35.0-35.4]), progression-free survival was improved in the tafa-len-R-CHOP group versus the R-CHOP group (hazard ratio [HR] 0.75 [95% CI 0.59-0.96]; $p=0.0194$), with 2-year progression-free survival rates of 71.1% (66.3-75.4) with tafa-len-R-CHOP versus 62.9% (57.9-67.5) with R-CHOP. Interim HR for overall survival was 0.85 (0.63-1.14). The overall rate of grade 3 or higher treatment-emergent adverse events was higher with tafa-len-R-CHOP (384 [87%] of 443) than with R-CHOP (340 [76%] of 447). Additionally, a higher rate of fatal treatment-emergent adverse events was observed with tafa-len-R-CHOP (26 [6%]) than with R-CHOP (17 [4%]). However, the number of overall deaths in the study was lower with tafa-len-R-CHOP than with R-CHOP (82 [19%] vs 97 [22%]). Based on disposition data, rates of premature discontinuation of all study drugs were similar in the tafa-len-R-CHOP group (71 [16%] of 443) and R-CHOP group (66 [15%] of 447).

INTERPRETATION: Progression-free survival was significantly improved with tafa-len-R-CHOP versus R-CHOP; however, the safety profile indicated increases in adverse events, including treatment-emergent adverse events leading to death, with the addition of tafasitamab and lenalidomide. Overall survival data are immature; follow-up is ongoing. Further analyses, including of circulating tumor DNA, will help to assess whether deeper molecular responses are contributing to the progression-free survival benefit observed with tafa-len-R-CHOP. Tafa-len-R-CHOP might represent a potential new first-line treatment for patients with high-risk DLBCL or HGBL.

FUNDING: Incyte Corporation.

Epcoritamab monotherapy or epcoritamab with lenalidomide as first-line therapy for patients with diffuse large B-cell lymphoma (EPCORE DLBCL-3): primary analysis of an open-label, multicentre, randomised, phase 2 trial.

Journal: The Lancet. Haematology | **Date:** 2026-Jul | **PMID:** 42285113 | **DOI:** 10.1016/S2352-3026(26)00112-2

Publication types: Journal Article, Randomized Controlled Trial, Multicenter Study, Clinical Trial, Phase II

Abstract:

BACKGROUND: Anthracycline-free regimens are needed for older adults with newly diagnosed diffuse large B-cell lymphoma (DLBCL). We aim to evaluate the efficacy and safety of fixed-duration epcoritamab monotherapy versus epcoritamab with lenalidomide in this patient population.

METHODS: This open-label, multicentre, randomised, phase 2 trial was conducted at 44 hospitals across 11 countries in Europe and Asia. Patients had newly diagnosed, histologically confirmed CD20-positive large B-cell lymphoma, were ineligible for anthracycline-based chemoimmunotherapy (age ≥ 80 or ≥ 75 years with clinically significant comorbidities), had Ann Arbor stage II-IV disease, and an Eastern Cooperative Oncology Group (ECOG) performance status of 0-2. In stage 1, patients were randomly assigned (1:1) to epcoritamab monotherapy or epcoritamab with lenalidomide using a validated interactive response technology system and stratified by the International Prognostic Index (<3 vs ≥ 3) and ECOG performance status (0-1 vs 2). Epcoritamab was administered subcutaneously using a two step-up dosing schedule (0.16 mg on cycle 1 day 1 and 0.8 mg on cycle 1 day 8), followed by full 48 mg doses once per week in 28-day cycles during cycles 1-3 and once every 4 weeks during cycles 4-12 for up to 12 cycles. Lenalidomide (10 or 20 mg) was given orally once a day on days 1-21 of 28-day cycles for up to 12 cycles. Based on stage 1, one of the treatment regimens was selected for expansion in stage 2. The primary endpoint was investigator-assessed complete response rate (as of the data cutoff on Dec 5, 2025; Lugano criteria) in the full analysis set (all randomly assigned patients in stage 1 and in all treated patients in stage 2). Safety was analysed in patients who received ≥ 1 dose of trial treatment. This study is registered with ClinicalTrials.gov, NCT05660967 (active, not recruiting).

FINDINGS: Between March 8, 2023, and May 14, 2025, 111 patients were assessed for eligibility (88 in stage 1 and 23 in stage 2) and 108 were treated. Median age was 83.0 years (IQR 80.0-86.0). 51 (47%) of 108 were male, 57 (53%) were female, and 87 (81%) were White. In stage 1, 44 patients each were randomly assigned to epcoritamab monotherapy or epcoritamab with lenalidomide and two did not receive treatment. At data cutoff, the complete response rate in stage 1 was 63.6% (95% CI 47.8-77.6; in 28 of 44 patients) in the epcoritamab monotherapy group and 45.5% (30.4-61.2; in 20 of 44) in the epcoritamab with lenalidomide group. The most common treatment-emergent adverse events (grade ≥ 3) were infections (eight [18%] of 44 patients) and fatigue (six [14%]) with epcoritamab monotherapy versus neutropenia (17 [40%] of 42) and infections (16 [38%]) with epcoritamab with lenalidomide; with serious adverse events in 28 (64%) versus 34 (81%). Treatment-emergent deaths occurred in six (14%) patients in the monotherapy group (cytomegalovirus infection reactivation, COVID-19 pneumonia, SARS-COV-2 infection, tumour lysis syndrome, neuroendocrine tumour of the lung, tumour haemorrhage) and six (14%) in the combined group (pneumonia, COVID-19 pneumonia, sepsis, general physical health deterioration, multiple organ dysfunction syndrome, acute cardiac failure). Based on differences in complete response rates and safety, epcoritamab monotherapy was selected for stage 2 and one patient did not receive treatment. In the epcoritamab monotherapy group, the complete response rate was 45.5% (24.4-67.8; in ten of 22) in stage 2 and 57.6% (44.8-69.7; in 38 of 66) across stages 1 and 2. The most common treatment-emergent adverse events (grade ≥ 3) among all patients who received epcoritamab monotherapy were infections (16 [24%]), neutropenia (eight [12%]), and hypertension (seven [11%]); with serious adverse events in 46 (70%). Treatment-emergent deaths occurred in two (3%) patients in stage 2 (multiple organ dysfunction syndrome and acute respiratory failure).

INTERPRETATION: Fixed-duration epcoritamab monotherapy showed promising complete response rates and a manageable safety profile in older adults with newly diagnosed DLBCL and comorbidities, with slight differences between stages 1 and 2. Continued investigation of epcoritamab as a first-line chemotherapy-free treatment option is warranted.

FUNDING: Genmab and AbbVie.

"Surveillance aid only. Verify every item against the primary source and apply clinical judgment and institutional protocols before any clinical use. Automated extraction can misread numbers, endpoints, and populations."